

SUNNAH REMEDIES ACADEMY

COMPLETE ASSESSMENT FRAMEWORK

SECTION 1: ASSESSMENT PHILOSOPHY

Core Principles

“Assessment as Learning, Not Just Assessment of Learning”

The Sunnah Remedies Academy assessment philosophy is built on three pillars:

1.1 Integration of Traditional Knowledge and Modern Standards

- Every assessment validates both traditional Sunnah-based healing knowledge and contemporary evidence-based practice
- Competency is measured across three domains: **Knowledge, Practice, and Spiritual Integration**
- Assessments mirror real-world clinical scenarios where Sunnah remedies are applied within modern healthcare contexts

1.2 Formative and Summative Balance

- **Formative (40% of total weight):** Continuous feedback, reflective practice, clinical logbooks, peer assessment
- **Summative (60% of total weight):** Written exams, practical assessments, OSCEs, portfolio review
- No single assessment determines pass/fail; progression requires consistent competency demonstration

1.3 Alignment with International Standards

The framework aligns with:

- **World Federation of Medical Education (WFME)** standards for competency-based assessment
 - **European Association of Traditional Chinese Medicine (ETCMA)** assessment frameworks
 - **NICE (UK) and Cochrane** evidence evaluation standards
 - **ISO 21001:2018** for educational organizations management systems
 - **Unani Medicine Council of India** and **TCM assessment frameworks** for traditional medicine integration
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SECTION 2: COMPETENCY FRAMEWORK

2.1 Core Competency Domains

Domain	Weight	Description
K1: Sunnah Knowledge	20%	Quranic verses, Hadith, Sunnah practices, historical context
K2: Medical Sciences	20%	Anatomy, physiology, pathology, pharmacology, nutrition
K3: Clinical Practice	25%	Patient assessment, treatment planning, remedy preparation
K4: Evidence & Research	15%	Critical appraisal, evidence evaluation, research methodology
K5: Ethics & Professionalism	10%	Islamic ethics, patient safety, informed consent, boundaries
K6: Communication & Reflection	10%	Patient communication, reflective practice, teaching ability

2.2 Competency Levels

Level	Description	Assessment Methods
Novice	Knowledge recall, guided practice	MCQs, structured observations
Competent	Application in standard situations	SAQs, OSCEs, case studies
Proficient	Independent practice, adaptation	Clinical assessments, complex case studies
Expert	Innovation, teaching, leadership	Portfolio, research projects, peer teaching

SECTION 3: WRITTEN EXAMINATIONS

3.1 Examination Types

3.1.1 Multiple Choice Questions (MCQs)

Structure:

- **Single Best Answer (SBA):** 60% of MCQ bank
- **Extended Matching Questions (EMQ):** 25% of MCQ bank
- **Multiple True/False (MTF):** 15% of MCQ bank

Design Principles:

- Each question tests application, not just recall
- Clinical vignettes precede 70% of questions
- Distractors reflect common misconceptions
- Questions map to specific learning outcomes and competency domains

Example MCQ Template:

Question ID: K2-PHYS-2024-001
Domain: K2 Medical Sciences

Sub-domain: Physiology

Level: Competent

Bloom's Level: Application

Clinical Vignette:

A 45-year-old patient presents with chronic fatigue.

Blood tests reveal iron deficiency anemia. The practitioner considers recommending black seed (*Nigella sativa*) supplementation alongside dietary modifications.

Question:

Which physiological mechanism best explains why black seed may support iron absorption in this patient?

- A. Increased gastric acid secretion enhancing non-heme iron solubility
- B. Direct chelation of iron increasing bioavailability
- C. Anti-inflammatory effects reducing intestinal mucosal damage
- D. Stimulation of erythropoietin production in the kidneys
- E. Inhibition of hepcidin expression improving iron export

Correct Answer: C

Distractor Analysis:

- A: Common misconception (black seed does not significantly increase acid)
- B: Incorrect mechanism (no chelation properties demonstrated)
- D: Overestimates effect (no direct erythropoietin stimulation)
- E: Too specific (hepcidin modulation not established)

Marking: +1 correct, 0 incorrect (no negative marking)

Time allocation: 1.5 minutes

MCQ Bank Specifications:

- Minimum 1,200 questions per certification level
- Annual review and update cycle
- Psychometric analysis: difficulty index (0.3-0.7), discrimination index (>0.2)
- 20% question renewal annually

3.1.2 Short Answer Questions (SAQs)

Structure:

- 8-12 questions per examination
- Each question has 2-4 parts (a, b, c, d)
- Mark range: 5-15 marks per question
- Total examination: 100 marks

Question Types:

- **Knowledge Application:** Apply Sunnah remedy knowledge to clinical scenario

- **Evidence Evaluation:** Critically appraise research evidence
- **Integration:** Connect traditional knowledge with modern science
- **Ethical Reasoning:** Navigate complex ethical situations

Example SAQ:

Question 2: Honey in Wound Management (15 marks)

A 62-year-old diabetic patient presents with a slow-healing ulcer on the lower leg. The patient expresses interest in using medical-grade honey as an adjunct treatment, referencing the Quranic mention of honey as healing.

- Outline the key bioactive components of medical-grade honey that contribute to wound healing. (4 marks)
- Explain the physiological mechanisms by which these components facilitate wound healing in diabetic ulcers. (5 marks)
- Discuss three precautions or contraindications specific to using honey in diabetic wound management. (3 marks)
- Describe how you would integrate honey therapy with conventional diabetic wound care while maintaining evidence-based practice. (3 marks)

Marking Scheme:

- Methylglyoxal (MGO) and its antibacterial properties (1)
 - Hydrogen peroxide production (1)
 - Low pH and osmotic effect (1)
 - Enzymes (glucose oxidase, catalase) and amino acids (1)
- Antibacterial action against MRSA and Pseudomonas (1)
 - Anti-inflammatory modulation of cytokines (1)
 - Autolytic debridement via osmotic effect (1)
 - Stimulation of fibroblast proliferation and angiogenesis (1)
 - Maintenance of moist wound environment (1)
- Blood glucose monitoring requirements (1)
 - Risk of allergic reaction in bee product-sensitive patients (1)
 - Need for medical-grade vs. raw honey distinction (1)
- Coordination with diabetes specialist (1)
 - Monitoring glycemic control during treatment (1)
 - Documentation and evidence tracking (1)

3.1.3 Long Case Studies (Written)

Structure:

- 2-3 complex cases per examination
- 90 minutes per case
- 50 marks per case

Case Design:

- Multi-layered patient presentations
- Integration of spiritual, physical, and social factors
- Require synthesis of multiple knowledge domains
- Include ethical dilemmas and resource constraints

3.2 Exam Paper Layout

3.2.1 Standard Paper Template

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SUNNAH REMEDIES ACADEMY
CERTIFICATION EXAMINATION
[Level: Practitioner / Advanced Practitioner / Specialist]

Module: [Module Name]
Date: [Date]
Time Allowed: [X] hours
Total Marks: [100/200]

INSTRUCTIONS TO CANDIDATES:

1. This paper consists of THREE sections:
   - Section A: Multiple Choice Questions (40 marks)
   - Section B: Short Answer Questions (40 marks)
   - Section C: Case Study/Clinical Reasoning (20 marks)

2. Answer ALL questions in ALL sections.

3. Write your candidate number clearly on each answer sheet.

4. For Section A, mark answers on the provided optical mark sheet.

5. For Sections B and C, write answers in the provided booklets.
   Additional paper is available on request.

6. All questions carry the marks indicated.

7. Scientific calculators are permitted. Mobile devices are prohibited.

8. Show all working in calculation questions.
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SECTION A: MULTIPLE CHOICE QUESTIONS (40 marks)

Instructions: Choose the SINGLE best answer for each question.
Each question is worth 1 mark. There is no negative marking.

Clinical Vignette 1:
[Scenario text...]

Question 1: [Question text...]

- A. [Option]
- B. [Option]
- C. [Option]
- D. [Option]
- E. [Option]

[Questions 1-40 follow...]

SECTION B: SHORT ANSWER QUESTIONS (40 marks)

Instructions: Answer ALL questions. Write clearly and concisely.
Use diagrams where appropriate.

Question 1 (10 marks):
[Question text with parts a, b, c...]

Question 2 (10 marks):
...

SECTION C: CLINICAL REASONING (20 marks)

Instructions: Read the case study carefully. Answer ALL parts.
Demonstrate clinical reasoning and evidence-based justification.

Case Study:
[Detailed clinical scenario...]

- (a) [Question requiring assessment and diagnosis] (5 marks)
- (b) [Question requiring treatment planning] (8 marks)
- (c) [Question requiring monitoring and follow-up] (4 marks)
- (d) [Question requiring ethical or professional consideration] (3 marks)

END OF EXAMINATION

3.3 Marking Schemes

3.3.1 Standard Marking Rubric

Mark Band	Descriptor	Characteristics
0-29%	Fail	Fundamental misunderstanding; dangerous errors; incomplete
30-39%	Marginal Fail	Some understanding but significant gaps; unsafe practice
40-49%	Pass (Borderline)	Basic competence; meets minimum standards; needs supervision
50-64%	Good Pass	Sound understanding; competent practice; some independence
65-79%	Merit	Strong understanding; confident practice; good judgment
80-100%	Distinction	Exceptional understanding; innovative thinking; leadership potential

3.3.2 Borderline Review Process

- All papers scoring 38-42% undergo second marking
- External examiner review for all borderline cases
- Holistic judgment considering: clinical safety, knowledge breadth, communication clarity

3.3.3 Standard Setting

- **Angoff Method:** Expert panel estimates probability of borderline candidate answering correctly
- **Borderline Regression:** Statistical analysis of candidate performance
- **Compensatory vs. Non-compensatory:** Some sections must be passed independently (e.g., patient safety questions)

SECTION 4: CLINICAL REASONING ASSESSMENTS

4.1 Clinical Reasoning Examination (CRE)

Format: Written, 3 hours, 100 marks

Structure:

- 4 complex cases (25 marks each)
- Progressive disclosure format (information released in stages)
- Candidates must demonstrate reasoning at each stage

Stages per Case:

1. **Initial Presentation (5 marks):** What information do you need? What are your differential diagnoses?
2. **Investigation and Assessment (8 marks):** What assessments would you conduct? How do you interpret findings?
3. **Treatment Planning (7 marks):** What is your evidence-based treatment plan? How do you integrate Sunnah remedies?

4. **Monitoring and Adaptation (5 marks):** How do you evaluate effectiveness? What are your contingency plans?

Clinical Reasoning Marking Criteria:

Criterion	Weight	Excellent (4)	Good (3)	Adequate (2)	Poor (1)
Information Gathering	20%	Systematic, comprehensive, prioritizes relevant data	Good coverage with minor omissions	Basic coverage, some relevant data missed	Unsystematic, misses critical information
Hypothesis Generation	25%	Comprehensive differentials, appropriate prioritization	Good differentials, reasonable prioritization	Limited differentials, poor prioritization	Inappropriate or dangerous hypotheses
Evidence Integration	25%	Seamlessly integrates traditional and modern evidence	Good integration with minor gaps	Attempts integration but inconsistent	No integration or inappropriate application
Decision Making	20%	Sound, safe, patient-centered, justifiable	Generally sound with minor concerns	Some concerns about safety or appropriateness	Unsafe or inappropriate decisions
Communication	10%	Clear, professional, empathetic, culturally sensitive	Good communication with minor issues	Adequate but lacks empathy or clarity	Unclear, unprofessional, or insensitive

4.2 Case Study Assessment

4.2.1 Case Study Types

Type 1: Standardized Patient Case (Written)

- Detailed patient history, examination findings, investigation results
- Candidates develop full management plan
- 2,000-3,000 word response

Type 2: Complex Multi-Factorial Case

- Patient with multiple comorbidities
- Social, spiritual, and cultural factors
- Resource constraints
- Ethical dilemmas

Type 3: Research Case Study

- Candidate analyzes published case study
- Critiques methodology and evidence

- Proposes improvements

4.2.2 Case Study Marking Rubric

Component	Marks	Criteria
Assessment and Diagnosis	20	Comprehensive assessment, accurate diagnosis, differential diagnoses
Evidence Review	20	Critical appraisal of relevant evidence, both traditional and modern
Treatment Plan	25	Evidence-based, safe, individualized, culturally appropriate
Implementation	15	Practical, feasible, considers resources and constraints
Monitoring and Evaluation	10	Clear metrics, appropriate follow-up, contingency plans
Ethical and Professional	10	Addresses ethical issues, maintains professionalism, informed consent

SECTION 5: OBJECTIVE STRUCTURED CLINICAL EXAMINATIONS (OSCE)

5.1 OSCE Station Design

5.1.1 Station Types

Station Type	Duration	Marks	Purpose
History Taking	10 min	10	Communication, information gathering
Physical Examination	10 min	10	Clinical skills, systematic approach
Practical Procedure	15 min	15	Technical competence, safety
Counseling and Education	10 min	10	Patient education, informed consent
Clinical Reasoning (Oral)	10 min	10	Thinking aloud, justification
Emergency Management	10 min	10	Urgent care, prioritization
Professionalism/Ethics	10 min	10	Ethical decision-making, boundaries
Integration Station	15 min	15	Synthesis of multiple skills

5.1.2 Standard OSCE Circuit

Standard Circuit (16 stations):

- 2 rest stations (5 minutes each)
- 14 active stations (10-15 minutes each)
- Total circuit time: 3.5 hours
- Total marks: 160

Advanced Circuit (20 stations):

- 2 rest stations
- 18 active stations
- Total circuit time: 4.5 hours
- Total marks: 200

5.2 OSCE Station Examples

Station 1: Hijama Consultation (History Taking)

Scenario: You are a Sunnah Remedies practitioner. A 34-year-old patient has been referred by their GP for Hijama therapy for chronic tension headaches. You have 10 minutes to take a focused history and determine suitability for Hijama.

Patient Brief (for actor):

- 34-year-old software engineer
- Chronic tension headaches for 3 years
- Interested in Hijama after hearing about it from a friend
- Takes paracetamol 2-3 times weekly
- No other medical conditions
- Works long hours, high stress
- Asks about “detox” benefits

Examiner Checklist:

Competency	Task	Marks
Introduction	Introduces self, explains role, gains consent	1
Chief Complaint	Elicits headache history (onset, duration, severity, pattern)	2
Medical History	Screens for contraindications (bleeding disorders, anticoagulants, diabetes, pregnancy)	2

Table 7 – continued

Competency	Task	Marks
Medication Review	Asks about all medications and supplements	1
Lifestyle Assessment	Explores stress, sleep, diet, hydration	1
Expectations	Manages expectations about “detox” claims	1
Hijama Specific	Explains procedure, aftercare, evidence base	1
Safety Screening	Identifies any red flags requiring GP referral	1
Communication	Empathetic, clear, culturally sensitive, professional	1

Station 2: Cupping Point Selection (Clinical Reasoning)

Scenario: You are preparing to perform Hijama on a patient with lower back pain. The patient has no contraindications. You have 15 minutes to:

1. Explain your point selection rationale
2. Demonstrate anatomical knowledge
3. Discuss evidence for your chosen points
4. Address safety considerations

Marking Domains:

- Anatomical accuracy (4 marks)
- Evidence-based justification (4 marks)
- Safety considerations (3 marks)
- Communication clarity (2 marks)
- Cultural sensitivity (2 marks)

Station 3: Emergency Response (Emergency Management)

Scenario: During a Hijama session, the patient suddenly becomes pale, dizzy, and reports feeling faint. You have 10 minutes to manage this situation.

Competencies Assessed:

- Recognition of vasovagal response (2 marks)
- Immediate safety actions (3 marks)
- Patient positioning and monitoring (2 marks)
- Communication with patient (2 marks)

- Documentation and follow-up (1 mark)

5.3 OSCE Quality Assurance

5.3.1 Standardization

- All patient actors trained using standardized scripts
- Examiners attend calibration sessions before each OSCE
- Global rating scales alongside checklists
- Video recording for quality review (with consent)

5.3.2 Examiner Training

- Mandatory 2-day training program
- Calibration exercises with video cases
- Inter-rater reliability testing (target: ICC > 0.8)
- Annual re-certification

SECTION 6: PRACTICAL ASSESSMENTS

6.1 Practical Competency Assessment (PCA)

6.1.1 Assessment Areas

Area	Competencies	Assessment Method
Remedy Preparation	Honey processing, herbal preparation, oil extraction	Direct observation, product evaluation
Hijama Technique	Point selection, skin preparation, cupping application, aftercare	OSCE, supervised practice
Cupping Therapy	Dry cupping, massage cupping, flash cupping	OSCE, video assessment
Patient Assessment	Pulse diagnosis, tongue diagnosis, constitutional assessment	OSCE, case-based assessment
Documentation	Clinical records, treatment plans, progress notes	Portfolio review
Safety Procedures	Infection control, emergency response, contraindication screening	OSCE, simulation

6.1.2 Direct Observation of Procedural Skills (DOPS)

DOPS Assessment Form:

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DIRECT OBSERVATION OF PROCEDURAL SKILLS (DOPS)

Candidate: _____ Assessor: _____

Date: _____ Procedure: _____

Setting: _____ Patient/Model: _____

COMPETENCY RATING SCALE:

- 1 = Unsatisfactory (unsafe, requires direct intervention)
- 2 = Below Standard (significant concerns, needs supervision)
- 3 = Borderline (minimum competence, needs continued supervision)
- 4 = Competent (safe to practice with indirect supervision)
- 5 = Excellent (independent practice, can teach others)

1. CONSENT AND EXPLANATION

- Obtained informed consent
- Explained procedure and rationale
- Discussed risks and benefits
- Checked understanding

Rating: [1] [2] [3] [4] [5] Comments: _____

2. PREPARATION

- Appropriate environment preparation
- Equipment check and sterilization
- Patient positioning and comfort
- Hand hygiene and PPE

Rating: [1] [2] [3] [4] [5] Comments: _____

3. TECHNICAL SKILL

- Anatomical knowledge demonstrated
- Precise and confident technique
- Appropriate duration and intensity
- Adaptation to patient response

Rating: [1] [2] [3] [4] [5] Comments: _____

4. SAFETY AND INFECTION CONTROL

- Sterile technique maintained
- Sharps safety (if applicable)
- Waste disposal appropriate
- Emergency preparedness

Rating: [1] [2] [3] [4] [5] Comments: _____

5. AFTERCARE AND DOCUMENTATION

- Clear aftercare instructions
- Patient comfort and monitoring
- Accurate documentation
- Follow-up plan communicated

Rating: [1] [2] [3] [4] [5] Comments: _____

6. PROFESSIONALISM AND COMMUNICATION

- Empathetic and respectful
- Clear communication throughout
- Cultural sensitivity demonstrated
- Time management

Rating: [1] [2] [3] [4] [5] Comments: _____

OVERALL ASSESSMENT:

- Not yet competent (requires further supervised practice)
- Competent (may practice with indirect supervision)
- Excellent (independent practice approved)

Assessor Signature: _____ Date: _____

Candidate Signature: _____ Date: _____

6.2 Practical Examination: Hijama Competency

6.2.1 Examination Structure

Part A: Pre-Procedure Assessment (30 minutes)

- Patient consultation and history taking
- Contraindication screening
- Treatment planning and point selection
- Informed consent process
- **Marks:** 30

Part B: Procedure Performance (45 minutes)

- Environment and equipment preparation
- Patient positioning and comfort
- Skin preparation and sterilization
- Cup application technique
- Bloodletting technique (if wet cupping)
- Monitoring and safety
- Aftercare and dressing
- **Marks:** 50

Part C: Post-Procedure (15 minutes)

- Patient education and aftercare instructions
- Documentation completion
- Follow-up planning
- **Marks:** 20

Total: 100 marks

6.2.2 Competency Rubric: Hijama Technique

Skill Component	Excellent (5)	Competent (4)	Borderline (3)	Below Standard (2)	Unsatisfactory (1)
Point Selection	Precise anatomical knowledge, evidence-based, individualized	Accurate, appropriate, some evidence	Basic accuracy, limited individualization	Inaccurate or inappropriate points	Dangerous point selection
Skin Preparation	Meticulous, sterile, patient comfort optimized	Thorough, sterile, comfortable	Adequate, minor issues	Inadequate sterile technique	Unsafe, risk of infection
Cup Application	Perfect technique, appropriate suction, patient monitoring	Good technique, appropriate suction	Adequate, minor adjustments needed	Poor technique, patient discomfort	Unsafe, risk of injury
Bloodletting	Precise, minimal trauma, appropriate volume, hemostasis	Good technique, appropriate volume	Adequate, minor concerns	Excessive trauma or volume	Unsafe, risk of significant bleeding
Aftercare	Comprehensive, clear, documented, follow-up arranged	Good instructions, documented	Basic instructions, documented	Inadequate instructions	No aftercare or documentation

SECTION 7: COMPETENCY RUBRICS

7.1 Generic Competency Rubric

7.1.1 Knowledge and Understanding

Level	Description	Indicators
1 - Novice	Limited knowledge, recall only	Can list basic facts; cannot explain mechanisms; makes errors in basic concepts
2 - Developing	Growing knowledge, some application	Can explain basic concepts; applies knowledge to simple cases; some gaps remain
3 - Competent	Sound knowledge, consistent application	Explains concepts accurately; applies to standard cases; recognizes limitations

Table 10 – continued

Level	Description	Indicators
4 - Proficient	Comprehensive knowledge, complex application	Integrates multiple concepts; applies to complex cases; adapts to novel situations
5 - Expert	Mastery, innovation, teaching	Synthesizes new knowledge; teaches others; contributes to field development

7.1.2 Clinical Skills

Level	Description	Indicators
1 - Novice	Requires constant guidance	Cannot perform without direct supervision; frequent errors; unsafe
2 - Developing	Requires close supervision	Performs with guidance; some errors; needs prompting for safety
3 - Competent	Performs with minimal supervision	Consistent performance; recognizes own limitations; seeks help appropriately
4 - Proficient	Independent, adapts to variation	Performs independently; adapts technique; anticipates problems
5 - Expert	Teaches, innovates, leads	Develops new techniques; teaches effectively; leads clinical development

7.1.3 Professionalism and Ethics

Level	Description	Indicators
1 - Unsatisfactory	Concerning behavior	Boundary violations; unprofessional conduct; ethical lapses
2 - Below Standard	Inconsistent professionalism	Occasional lapses; requires reminders; limited insight
3 - Developing	Generally professional	Usually appropriate; some inconsistency; growing awareness
4 - Competent	Consistently professional	Reliable professionalism; good boundaries; ethical reasoning
5 - Exemplary	Role model	Exemplary conduct; mentors others; advocates for profession

7.2 Domain-Specific Rubrics

7.2.1 Sunnah Knowledge Integration

Criterion	1 (Poor)	2 (Below Standard)	3 (Competent)	4 (Good)	5 (Excellent)
Quranic Reference	Inaccurate or inappropriate references	Basic references with errors	Accurate references, basic context	Accurate with historical and linguistic context	Comprehensive, nuanced, scholarly
Hadith Authentication	Uses weak/fabricated hadith without awareness	Uses weak hadith with some awareness	Uses authenticated hadith appropriately	Critically evaluates hadith authenticity	Scholarly analysis of chains and narrators
Historical Context	No historical understanding	Basic historical awareness	Good understanding of historical practice	Comprehensive historical and cultural context	Original research contribution
Modern Application	Inappropriate or unsafe application	Basic application with concerns	Safe, appropriate application	Innovative, evidence-informed application	Pioneering integration, research-backed

7.2.2 Evidence-Based Practice

Criterion	1 (Poor)	2 (Below Standard)	3 (Competent)	4 (Good)	5 (Excellent)
Literature Search	Cannot locate relevant evidence	Basic search, misses key studies	Adequate search, most relevant studies found	Comprehensive search, including grey literature	Systematic, exhaustive, innovative sources
Critical Appraisal	Cannot evaluate evidence quality	Basic evaluation, significant errors	Adequate appraisal, recognizes major flaws	Good appraisal, understands study design and bias	Expert appraisal, identifies subtle methodological issues
Evidence Integration	Ignores evidence or misapplies it	Basic integration, some errors	Appropriate integration with traditional knowledge	Seamless integration, balanced approach	Innovative synthesis, contributes to evidence base
Clinical Application	Evidence not applied to practice	Basic application, some concerns	Evidence informs clinical decisions	Evidence fully integrated into personalized care	Evidence drives innovation and best practice

SECTION 8: FEEDBACK SYSTEMS

8.1 Formative Feedback

8.1.1 Continuous Feedback Model

After Every Assessment:

- Written feedback within 5 working days
- Strengths and areas for improvement identified
- Specific action points for development
- Reference to learning resources

Feedback Template:

SUNNAH REMEDIES ACADEMY
ASSESSMENT FEEDBACK FORM

Candidate: _____

Assessment: _____

Date: _____

Assessor: _____

OVERALL PERFORMANCE: [] Pass [] Fail [] Borderline

STRENGTHS DEMONSTRATED:

1. _____
2. _____
3. _____

AREAS FOR DEVELOPMENT:

1. _____
Action: _____
Resources: _____
2. _____
Action: _____
Resources: _____
3. _____
Action: _____
Resources: _____

CLINICAL SAFETY ASSESSMENT:

- No safety concerns identified
- Minor safety concerns - supervision required
- Significant safety concerns - direct supervision mandatory
- Critical safety concerns - immediate intervention required

Details: _____

PROGRESSION RECOMMENDATION:

- Progress to next stage
- Repeat assessment after remedial work
- Additional supervised practice required
- Refer to academic support services

Assessor Signature: _____ Date: _____

Candidate Signature: _____ Date: _____

8.1.2 Peer Feedback System

Peer Assessment Components:

- Group project contributions
- Peer teaching sessions
- Clinical observation feedback
- Professional behavior

Peer Feedback Form:

PEER FEEDBACK FORM

Your Name: _____ Peer Name: _____

Date: _____ Context: _____

Please rate your peer on the following (1-5 scale):

1. Knowledge and Preparation: [1] [2] [3] [4] [5]
2. Communication Clarity: [1] [2] [3] [4] [5]
3. Professionalism: [1] [2] [3] [4] [5]
4. Collaboration: [1] [2] [3] [4] [5]
5. Cultural Sensitivity: [1] [2] [3] [4] [5]

What did this peer do well?

What could this peer improve?

Any other comments?

8.2 Summative Feedback

8.2.1 Examination Results Feedback

Comprehensive Results Report:

- Overall score and grade
- Performance by domain and competency
- Comparison to cohort (anonymized)

- Specific question/item feedback (where appropriate)
- Development recommendations

8.2.2 Failure Feedback Protocol

Mandatory Components:

1. Face-to-face feedback session within 10 working days
2. Detailed analysis of failure points
3. Personalized remedial plan
4. Academic support referral
5. Reassessment timeline and conditions

SECTION 9: REFLECTIVE ASSIGNMENTS

9.1 Reflective Practice Portfolio

9.1.1 Structure

Mandatory Reflective Entries:

Entry	Timing	Focus	Word Count
Entry 1	Month 1	Initial expectations and learning goals	500
Entry 2	Month 3	First clinical experience reflection	1,000
Entry 3	Month 6	Mid-point review and adaptation	1,000
Entry 4	Month 9	Complex case reflection	1,500
Entry 5	Month 12	Final synthesis and future planning	1,500

9.1.2 Reflective Writing Framework

Gibbs’ Reflective Cycle (Adapted for Sunnah Remedies):

REFLECTIVE WRITING FRAMEWORK

1. DESCRIPTION (What happened?)

- Objective account of the experience

- Context: patient, setting, your role

- Key events in chronological order

2. FEELINGS (What were you thinking and feeling?)

- Initial thoughts and emotions

- How did you feel during the experience?

- How did you feel after the experience?
- Spiritual reflections (if applicable)

3. EVALUATION (What was good and bad about the experience?)

- What went well?
- What challenges did you face?
- What would you do differently?
- How did Sunnah principles guide your practice?

4. ANALYSIS (What sense can you make of the situation?)

- Why did things happen as they did?
- What knowledge informed your actions?
- What evidence supported your decisions?
- How did traditional and modern knowledge integrate?

5. CONCLUSION (What else could you have done?)

- What have you learned about yourself?
- What have you learned about your practice?
- What gaps in knowledge or skill have you identified?

6. ACTION PLAN (What would you do if it arose again?)

- Specific, measurable actions
- Resources needed
- Timeline for implementation
- How will you evaluate improvement?

9.1.3 Reflective Assignment Marking Rubric

Criterion	Weight	Excellent (5)	Good (4)	Competent (3)	Developing (2)	Poor (1)
Description	15%	Vivid, objective, comprehensive	Clear, detailed	Adequate description	Vague or incomplete	Missing or inaccurate
Reflection Depth	25%	Profound, transformative insight	Genuine insight, some depth	Surface reflection, some insight	Descriptive rather than reflective	No reflection evident
Critical Analysis	25%	Sophisticated analysis, multiple perspectives	Good analysis, evidence-based	Basic analysis, some evidence	Limited analysis, no evidence	No analysis, purely descriptive
Integration	20%	Seamless integration of Sunnah and modern knowledge	Good integration, some gaps	Attempts integration	No integration	Contradict or inappropriate
Action Plan	15%	Specific, measurable, achievable, realistic, time-bound	Good plan, mostly SMART	Basic plan, some specificity	Vague plan, not actionable	No plan or unrealistic

9.2 Critical Incident Analysis

Requirements:

- 2,000-word analysis of a significant clinical incident
 - Must include patient safety considerations
 - Evidence-based analysis of decision-making
 - Ethical dimensions explored
 - Personal and professional learning identified
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SECTION 10: PORTFOLIO ASSESSMENT

10.1 Portfolio Structure

10.1.1 Portfolio Components

Section A: Evidence of Learning (40%)

Component	Description	Evidence Required
Learning Log	Record of all learning activities	Dates, hours, reflections
Workshop Certificates	Attendance at mandatory workshops	Certificates, reflections
Online Learning	Completed e-learning modules	Completion certificates, quiz scores
Reading Record	Evidence of scholarly reading	Annotated bibliography, critical reviews
Peer Teaching	Evidence of teaching others	Session plans, evaluations, reflections

Section B: Clinical Evidence (40%)

Component	Description	Evidence Required
Clinical Logbook	Record of all patient encounters	De-identified case records
Case Studies	Detailed analysis of 5 complex cases	Full case write-ups with evidence
DOPS Assessments	Directly observed procedural skills	Minimum 10 DOPS forms
Mini-CEX	Mini clinical evaluation exercises	Minimum 5 Mini-CEX forms
Patient Feedback	Anonymous patient satisfaction data	Minimum 20 feedback forms

Section C: Professional Development (20%)

Component	Description	Evidence Required
Reflective Journal	Ongoing reflective practice	12 entries minimum
CPD Record	Continuing professional development	Certificates, reflections
Research Engagement	Evidence of research participation	Project involvement, publications
Professional Activities	Conference attendance, committee work	Certificates, reports
Mentorship Record	Evidence of being mentored and mentoring	Meeting records, reflections

10.1.2 Portfolio Marking Rubric

Domain	Weight	Excellent (5)	Good (4)	Competent (3)	Developing (2)	Inadequate (1)
Completeness	25%	All components present, exceeds requirements	All components present, meets requirements	Most components present	Several components missing	Many components missing
Quality	25%	Exceptional quality, publishable standard	High quality, professional standard	Adequate quality, minor issues	Variable quality, some concerns	Poor quality, significant concerns
Reflection	20%	Deep, transformative reflection throughout	Good reflection, most entries	Adequate reflection, some entries	Limited reflection	No reflection evident
Evidence	20%	Comprehensive, well-organized, cross-referenced	Good evidence, well-organized	Adequate evidence, some organization	Limited evidence, poorly organized	Insufficient evidence
Integration	15%	Seamless integration of theory and practice	Good integration, some gaps	Basic integration	Limited integration	No integration

10.2 Mini-Clinical Evaluation Exercise (Mini-CEX)

10.2.1 Mini-CEX Form

SUNNAH REMEDIES ACADEMY

MINI-CLINICAL EVALUATION EXERCISE (Mini-CEX)

Candidate: _____ Assessor: _____

Date: _____ Setting: _____

Patient Type: _____ Complexity: [] Low [] Medium [] High
Time Observed: _____ minutes

Please rate the candidate's performance (1-9 scale):

1-3 = Unsatisfactory (significant concerns)

4-6 = Borderline (needs development)

7-9 = Satisfactory to Excellent

1. HISTORY TAKING

[1] [2] [3] [4] [5] [6] [7] [8] [9]

Comments: _____

2. PHYSICAL EXAMINATION

[1] [2] [3] [4] [5] [6] [7] [8] [9]

Comments: _____

3. CLINICAL JUDGMENT

[1] [2] [3] [4] [5] [6] [7] [8] [9]

Comments: _____

4. COMMUNICATION SKILLS

[1] [2] [3] [4] [5] [6] [7] [8] [9]

Comments: _____

5. PROFESSIONALISM

[1] [2] [3] [4] [5] [6] [7] [8] [9]

Comments: _____

6. OVERALL CLINICAL CARE

[1] [2] [3] [4] [5] [6] [7] [8] [9]

Comments: _____

FEEDBACK TO CANDIDATE:

What was done well:

Areas for improvement:

Specific action points:

Assessor Signature: _____ Date: _____

Candidate Signature: _____ Date: _____

SECTION 11: CLINICAL LOGBOOKS

11.1 Clinical Encounter Logbook

11.1.1 Logbook Requirements

Minimum Case Requirements by Certification Level:

Level	Total Cases	Supervised Cases	Independent Cases	Complex Cases
Foundation	50	50	0	5
Practitioner	150	100	50	20
Advanced	300	150	150	50
Specialist	500	200	300	100

11.1.2 Case Documentation Template

CLINICAL ENCOUNTER RECORD		
Case ID: _____	Date: _____	
Candidate: _____	Supervisor: _____	
Setting: _____	Patient Age/Sex: _____	
PRESENTING COMPLAINT:		

HISTORY OF PRESENTING COMPLAINT:		

RELEVANT MEDICAL HISTORY:		

CURRENT MEDICATIONS:		

SUNNAH REMEDIES CONSIDERED:		
- Hijama (Wet)	- Hijama (Dry)	- Cupping (Massage)
- Honey Therapy	- Black Seed	- Olive Oil
- Miswak	- Dates	- Other: _____
ASSESSMENT FINDINGS:		

TREATMENT PROVIDED:		

EVIDENCE BASE:		

- Quranic reference: _____
- Hadith reference: _____
- Clinical evidence: _____
- Traditional knowledge: _____

OUTCOME:

- Improved - Unchanged - Deteriorated - Unable to assess

Details: _____

REFLECTION:

What did you learn from this case?

What would you do differently?

SUPERVISOR COMMENTS:

Supervisor Signature: _____ Date: _____

Candidate Signature: _____ Date: _____

11.2 Procedure Logbook

11.2.1 Hijama Procedure Log

Date	Patient ID	Type	Points	Volume	Supervisor	Complications	Outcome
		Wet/ Dry					

Minimum Procedure Requirements:

- Wet Hijama: 50 supervised, 50 independent
- Dry Cupping: 30 supervised, 30 independent
- Massage Cupping: 20 supervised, 20 independent

SECTION 12: PROGRESS TRACKING

12.1 Learner Progress Dashboard

12.1.1 Progress Tracking System

Individual Progress Record:

Competency Domain	Foundation	Practitioner	Advanced	Specialist	Current Status
K1: Sunnah Knowledge	[]	[]	[]	[]	[Progress %]
K2: Medical Sciences	[]	[]	[]	[]	[Progress %]
K3: Clinical Practice	[]	[]	[]	[]	[Progress %]
K4: Evidence & Research	[]	[]	[]	[]	[Progress %]
K5: Ethics & Professionalism	[]	[]	[]	[]	[Progress %]
K6: Communication & Reflection	[]	[]	[]	[]	[Progress %]

Assessment Progress:

Assessment Type	Required	Completed	Pass/Fail	Date	Next Due
MCQ Examination	4				
SAQ Examination	4				
OSCE Practical Assessment	3				
Case Studies	5				
Portfolio Review	4				
Reflective Assignments	12				

12.1.2 Milestone Reviews

Review Schedule:

- **Monthly:** Supervisory meetings, case review
- **Quarterly:** Progress review against competencies, portfolio check
- **Bi-annually:** Formal assessment review, progression decision
- **Annually:** Comprehensive review, certification decision

Progression Criteria:

- All mandatory assessments completed
- Minimum 50% score in all competency domains
- No critical safety concerns
- Portfolio complete and satisfactory
- Supervisor recommendation

12.2 Competency Tracker

12.2.1 Visual Competency Map



SECTION 13: CERTIFICATION REQUIREMENTS

13.1 Certification Levels

13.1.1 Foundation Certificate in Sunnah Remedies

Duration: 6 months **Entry Requirements:**

- High school diploma or equivalent
- Basic understanding of Islamic principles
- First aid certification

Assessment Requirements:

Component	Weight	Pass Mark	Required
MCQ Examination	30%	60%	1
SAQ Examination	20%	60%	1
Practical Assessment	30%	70%	1
Portfolio	10%	Pass	1
Reflective Assignments	10%	Pass	3

Competency Thresholds:

- All safety-critical competencies: 80% minimum
- Knowledge domains: 60% minimum
- Clinical skills: 70% minimum

13.1.2 Practitioner Certificate in Sunnah Remedies

Duration: 12 months (following Foundation) **Entry Requirements:**

- Foundation Certificate
- 50 supervised clinical hours
- Supervisor recommendation

Assessment Requirements:

Component	Weight	Pass Mark	Required
Written Examinations (MCQ + SAQ)	25%	65%	2
OSCE	25%	70%	2
Practical Assessment	25%	75%	2
Portfolio	15%	Pass	1
Case Studies	10%	65%	3

Clinical Requirements:

- 150 total patient encounters
- 100 supervised procedures
- 50 independent procedures
- 20 complex cases

13.1.3 Advanced Practitioner Certificate

Duration: 18 months (following Practitioner) **Entry Requirements:**

- Practitioner Certificate
- 12 months clinical practice
- 300 clinical hours logged

Assessment Requirements:

Component	Weight	Pass Mark	Required
Written Examinations	20%	70%	2
OSCE	20%	75%	2
Practical Assessment	20%	80%	2
Portfolio	15%	Pass	1
Research Project	15%	65%	1
Case Studies	10%	70%	5

Additional Requirements:

- Lead a quality improvement project
- Mentor 2 Foundation students
- Present at a professional meeting
- Publish a case study or article

13.1.4 Specialist Certificate

Duration: 24 months (following Advanced) **Entry Requirements:**

- Advanced Practitioner Certificate
- 24 months clinical practice
- 500 clinical hours logged

Assessment Requirements:

Component	Weight	Pass Mark	Required
Written Examinations	15%	75%	2
OSCE	15%	80%	2
Practical Assessment	15%	85%	2
Portfolio	15%	Pass	1
Research Project	20%	70%	1
Teaching Assessment	10%	75%	1
Viva Voce	10%	75%	1

Additional Requirements:

- Original research contribution
- Teach at least 2 workshops
- Develop a clinical guideline
- Peer review 5 case studies

13.2 Certification Renewal

13.2.1 Continuing Professional Development (CPD)

Annual Requirements:

- 40 hours CPD (minimum 20 hours formal)
- 2 peer-reviewed case discussions
- 1 reflective assignment
- Patient feedback review
- Supervisor review

13.2.2 Re-certification Assessment

Every 3 Years:

- Portfolio review
- Updated clinical logbook (minimum 100 cases)
- CPD record

- Peer and patient feedback
- Viva voce (if concerns identified)

SECTION 14: QUALITY ASSURANCE

14.1 Assessment Validity and Reliability

14.1.1 Psychometric Standards

Property	Standard	Monitoring Method
Content Validity	Expert panel review, blueprint alignment	Annual blueprint review
Construct Validity	Factor analysis, correlation with other measures	Statistical analysis post-examination
Criterion Validity	Correlation with clinical performance	Longitudinal tracking
Reliability	Cronbach' s alpha > 0.8 for written exams	Post-examination analysis
Inter-rater Reliability	ICC > 0.8 for practical assessments	Regular calibration sessions
Standard Error of Measurement	< 5% of total score	Statistical monitoring

14.1.2 Standard Setting

Angoff Method Process:

1. Expert panel identifies “borderline” candidate
2. Each expert estimates probability of borderline candidate answering correctly
3. Mean probability becomes cut score
4. Statistical validation against candidate performance

Borderline Regression:

- Regression analysis of station scores against global judgment
 - Identifies borderline candidates for holistic review
-

14.2 Examiner Quality Assurance

14.2.1 Examiner Requirements

- Minimum 5 years clinical experience
- Current certification at appropriate level
- Completion of examiner training program

- Annual calibration and re-certification
- Peer review of marking (minimum 10% double-marked)

14.2.2 Examiner Training Program

Module 1: Assessment Principles (4 hours)

- Validity, reliability, fairness
- Standard setting methods
- Bias awareness and mitigation

Module 2: Marking Calibration (4 hours)

- Standardized marking exercises
- Inter-rater reliability testing
- Feedback and discussion

Module 3: OSCE Conduct (4 hours)

- Standardized patient training
- Examiner behavior and consistency
- Giving feedback

Module 4: Quality Assurance (2 hours)

- Monitoring and evaluation
 - Appeals handling
 - Continuous improvement
-

14.3 Appeals and Complaints

14.3.1 Appeals Process

Stage 1: Informal Resolution

- Candidate discusses concern with assessor
- Resolution within 5 working days

Stage 2: Formal Appeal

- Written submission to Assessment Board
- Independent review of assessment materials
- Decision within 15 working days

Stage 3: External Review

- Independent external examiner review
- Final decision within 30 working days

14.3.2 Grounds for Appeal

- Procedural irregularity
 - Administrative error
 - Bias or discrimination
 - Extenuating circumstances (not previously declared)
-

SECTION 15: SPECIAL CONSIDERATIONS

15.1 Reasonable Adjustments

15.1.1 Principles

- Equal opportunity to demonstrate competence
- No compromise to assessment standards
- Individualized assessment of needs

15.1.2 Common Adjustments

Need	Adjustment	Implementation
Dyslexia	Extra time (25%), separate room, reader	Confirmed by educational psychologist
Visual Impairment	Enlarged papers, screen reader, scribe	Confirmed by ophthalmologist
Hearing Impairment	Lip-reader, sign interpreter, written instructions	Confirmed by audiologist
Physical Disability	Accessible venue, adjusted practical tasks	Confirmed by medical assessment
Anxiety/ Mental Health	Separate room, rest breaks, deferral	Confirmed by mental health professional
Religious Observance	Alternative dates, prayer breaks, halal materials	Confirmed by religious authority

15.2 Malpractice and Integrity

15.2.1 Academic Integrity Policy

Prohibited Conduct:

- Plagiarism in any form
- Collusion on individual assessments
- Impersonation
- Use of unauthorized materials
- Fabrication of clinical records

- Falsification of portfolio evidence

15.2.2 Consequences

Offense	First Instance	Second Instance	Third Instance
Minor (poor referencing)	Warning, remedial training	Zero mark, resubmission	Disciplinary panel
Moderate (collusion)	Zero mark, resubmission	Module fail, academic probation	Exclusion
Major (impersonation, fabrication)	Module fail, disciplinary panel	Exclusion	Permanent exclusion

SECTION 16: APPENDICES

Appendix A: Assessment Blueprint Template

ASSESSMENT BLUEPRINT

Certification Level: _____

Assessment Type: _____

Date: _____

DOMAIN DISTRIBUTION:

K1: Sunnah Knowledge (20%)

- Quranic healing references: []%
- Hadith authentication: []%
- Historical practice: []%
- Modern application: []%

K2: Medical Sciences (20%)

- Anatomy and physiology: []%
- Pathology: []%
- Pharmacology: []%
- Nutrition: []%

K3: Clinical Practice (25%)

- Assessment skills: []%
- Treatment planning: []%
- Procedure skills: []%
- Patient management: []%

K4: Evidence and Research (15%)

- Critical appraisal: []%
- Evidence integration: []%
- Research methodology: []%

K5: Ethics and Professionalism (10%)

- Islamic ethics: []%
- Patient safety: []%
- Professional boundaries: []%

K6: Communication and Reflection (10%)

- Patient communication: []%
- Documentation: []%
- Reflective practice: []%

COGNITIVE LEVEL DISTRIBUTION:

Remembering (Bloom 1): []%

Understanding (Bloom 2): []%

Applying (Bloom 3): []%

Analyzing (Bloom 4): []%

Evaluating (Bloom 5): []%

Creating (Bloom 6): []%

ITEM TYPE DISTRIBUTION:

MCQ (Single Best Answer): [] questions

MCQ (Extended Matching): [] questions

SAQ (5 marks): [] questions

SAQ (10 marks): [] questions

SAQ (15 marks): [] questions

Case Study: [] cases

OSCE Stations: [] stations

Blueprint approved by: _____ Date: _____

Appendix B: Exam Paper Checklist

Pre-Examination:

- Blueprint completed and approved
- Questions written by subject matter experts
- Questions reviewed by second examiner
- Questions checked for bias and cultural sensitivity
- Standard setting completed
- Materials prepared and proofread
- Venue booked and accessibility checked
- Examiners briefed and calibrated

During Examination:

- Candidate identity verified

- Instructions read clearly
- Timing announced
- Invigilation maintained
- Security protocols followed

Post-Examination:

- Papers collected and secured
- Marking completed within timeframe
- Double-marking for borderline cases
- Standard error calculated
- Results checked for errors
- Feedback prepared
- Results released within 15 working days

Appendix C: OSCE Station Template

OSCE STATION TEMPLATE

Station Number: _____

Station Type: _____

Duration: _____ minutes

Marks: _____

STATION TITLE: _____

COMPETENCIES ASSESSED:

1. _____
2. _____
3. _____

CANDIDATE INSTRUCTIONS:

[Write clear, concise instructions for the candidate]

PATIENT ACTOR BRIEF:

[Detailed brief for the standardized patient, including:

- Background and history
- Emotional state and behavior
- Questions to ask
- Responses to give
- Cues to provide]

EXAMINER CHECKLIST:

Competency	Task	Marks

-----|-----|-----|
| | | |
| | | |
| | | |

GLOBAL RATING:

[1] Unsatisfactory [2] Borderline [3] Competent [4] Good [5] Excellent

EQUIPMENT REQUIRED:

- _____
- _____

Appendix D: Assessment Calendar Template

Month	Week	Assessment Activity	Responsible	Venue
1	1	Foundation MCQ	Assessment Team	Exam Hall
1	3	Foundation SAQ	Assessment Team	Exam Hall
2	2	Foundation Practical	Clinical Team	Clinic
3	1	Mid-term OSCE	OSCE Team	OSCE Centre
3	3	Portfolio Review 1	Portfolio Panel	Meeting Room
6	1	Practitioner MCQ	Assessment Team	Exam Hall
6	2	Practitioner SAQ	Assessment Team	Exam Hall
6	4	Practitioner OSCE	OSCE Team	OSCE Centre
7	2	Practical Assessment	Clinical Team	Clinic
9	1	Case Study Submission	Assessment Team	Online
9	3	Portfolio Review 2	Portfolio Panel	Meeting Room
12	1	Final OSCE	OSCE Team	OSCE Centre
12	2	Final Practical	Clinical Team	Clinic
12	4	Viva Voce (Specialist)	Exam Panel	Meeting Room

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This Assessment Framework establishes Sunnah Remedies Academy as a world-class institution that honors traditional Islamic healing wisdom while meeting rigorous international healthcare education standards. Every assessment is designed not merely to test knowledge, but to cultivate practitioners who embody the prophetic tradition of compassionate, evidence-based, spiritually-grounded healing.